Thyrocare

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Name : Nishant Kumar(29Y/M)

Date : 13 Apr, 2026

Test Asked : Aarogyam Camp Profile 2

Report Status : Complete Report

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Patient Name	: **NISHANT KUMAR (29Y/M)**	Tests Done	: **AAROGYAM CAMP PROFILE 2**
Referred By	: **SELF**		
Address	: **PLOOT NO- 292, BARI CO-OPERATIVE COLONY, SIWANDIH, BOKARO STEEL CITY, JHARKHAND - 827010**		

Report Availability Summary

Note: Please refer to the table below for status of your tests.

10 Ready | **0** Ready with Cancellation | **0** Processing | **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
AAROGYAM CAMP PROFILE 2	Ready
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	Ready
Lipoprotein (a) [Lp(a)]	Ready
HBA PROFILE	Ready
HEMOGRAM - 6 PART (DIFF)	Ready
LIVER FUNCTION TESTS	Ready
KIDPRO	Ready
LIPID PROFILE	Ready
T3-T4-USTSH	Ready
VITAMIN D TOTAL AND B12 COMBO	Ready
APOLIPROTEIN RATIO	Ready

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Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
CARDIAC RISK MARKERS			
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	**3.98**	mg/L	< 3
COMPLETE HEMOGRAM			
HEMATOCRIT(PCV)	**52.7**	%	40.0-50.0
MEAN CORP.HEMO.CONC(MCHC)	**27.5**	g/dL	31.5-34.5
MEAN CORPUSCULAR HEMOGLOBIN(MCH)	**25.9**	pq	27.0-32.0
MEAN PLATELET VOLUME(MPV)	**13**	fL	6.5-12
MONOCYTES	**1.8**	%	2-10
MONOCYTES - ABSOLUTE COUNT	**0.12**	X 10^3 / µL	0.2 - 1.0
PLATELET DISTRIBUTION WIDTH(PDW)	**19**	fL	9.6-15.2
PLATELET TO LARGE CELL RATIO(PLCR)	**48.4**	%	19.7-42.4
RED CELL DISTRIBUTION WIDTH (RDW-CV)	**16.2**	%	11.6-14
RED CELL DISTRIBUTION WIDTH - SD(RDW-SD)	**56.4**	fL	39-46
TOTAL RBC	**5.59**	X 10^6/µL	4.5-5.5
LIPID			
HDL CHOLESTEROL - DIRECT	**26**	mg/dL	40-60
TRIG / HDL RATIO	**6.74**	Ratio	< 3.12
TRIGLYCERIDES	**174**	mg/dL	< 150
LIVER			
ASPARTATE AMINOTRANSFERASE (SGOT)	**41.3**	U/L	< 35
SERUM GLOBULIN	**3.63**	gm/dL	2.5-3.4
VITAMINS			
25-OH VITAMIN D (TOTAL)	**5.31**	ng/mL	30-100
VITAMIN B-12	**160**	pg/mL	211-911

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

Patient Name	: NISHANT KUMAR (29Y/M)	**Sample Collected on (SCT)**	: 13 Apr 2026 11:52
Referred By	: SELF	**Sample Received on (SRT)**	: 13 Apr 2026 22:05
Address	: PLOOT NO- 292, BARI CO-OPERATIVE COLONY, SIWANDIH, BOKARO STEEL CITY, JHARKHAND - 827010	**Report Released on (RRT)**	: 14 Apr 2026 01:13
		Sample Type | Barcode	: SERUM | EV175169

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	**C.L.I.A**	**5.31**	**ng/mL**

Bio. Ref. Interval. :
DEFICIENCY : <20 ng/ml || INSUFFICIENCY : 20-<30 ng/ml
SUFFICIENCY : 30-100 ng/ml || TOXICITY : >100 ng/ml

Clinical Significance:

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health. Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome. Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml.

Kit Validation Reference: Holick MF. Vitamin D Deficiency. N Engl J Med. 2007;357:266–81.

Method : Fully Automated Chemi Luminescent Immuno Assay

VITAMIN B-12	**C.L.I.A**	**160**	**pg/mL**

Bio. Ref. Interval. :

Normal : 211 - 911 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):5.0%, Inter assay (%CV):9.2 %;Sensitivity:45 pg/ml

Kit Validation reference:
Chen IW, Sperling MI, Heminger LA. Vitamin B12. In: Pesce AJ, Kaplan LA, eds. Methods in Clinical Chemistry. St. Louis: CV Mosby; 1987:569–73.

Method : COMPETITIVE CHEMI LUMINESCENT IMMUNO ASSAY

Please correlate with clinical conditions.

Tests Done : AAROGYAM CAMP PROFILE 2

Report Remarks : Labcode:1304111774/JRD32

Dr Anupama Sinha
MD(Path)

Dr.Amit Anand
DCP(Path)

Scan QR to verify(valid for 30 days from release time)

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TEST NAME	TECHNOLOGY	VALUE	UNITS
APOLIPOPROTEIN - A1 (APO-A1)	IMMUNOTURBIDIMETRY	94	mg/dL

Bio. Ref. Interval. :
Male : 86 - 152
Female : 94 - 162

Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

APOLIPOPROTEIN - B (APO-B)	IMMUNOTURBIDIMETRY	59	mg/dL

Bio. Ref. Interval. :
Male : 56 - 145
Female : 53 - 138

Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

APO B / APO A1 RATIO (APO B/A1)	CALCULATED	0.6	Ratio

Bio. Ref. Interval. :

Male : 0.40 - 1.26
Female : 0.38 - 1.14

Clinical Significance :

- Apolipoprotein B is a more potent and independent predictor of Coronary artery disease (CAD) than LDL Cholesterol.
- Apolipoprotein A1 is one of the apoproteins of HDL and is inversely related to risk of CAD.
- The Apolipoprotein studies help in monitoring risk of restenosis in patients with myocardial infarction, Coronary bypass surgery etc.
- An increased ratio of Apo B to A1 beyond the defined normal range is indicative of CAD risk.
- All results have to be interpreted in Conjunction with clinical history and other findings.

Method : Derived from serum Apo A1 and Apo B values

Please correlate with clinical conditions.

Tests Done : AAROGYAM CAMP PROFILE 2

Report Remarks : Labcode:1304111774/JRD32

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MD(Path)

Dr.Amit Anand
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TEST NAME	TECHNOLOGY	VALUE	UNITS
LIPOPROTEIN (A) [LP(A)]	IMMUNOTURBIDIMETRY	3.4	mg/dL

Bio. Ref. Interval. :-

Adults : < 30.0 mg/dl

Clinical Significance:
Determination of LPA may be useful to guide management of individuals with a family history of CHD or with existing disease. The levels of LPA in the blood depends on genetic factors; The range of variation in a population is relatively large and hence for diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:
Precision %CV :- Intra assay %CV- 4.55% , Inter assay %CV-0.86 %

Kit Validation Reference:
Tietz NW,Clinical Guide to Laboratory Tests Philadelphia WB. Saunders 1995 : 442-444

Please correlate with clinical conditions.

Method:- LATEX ENHANCED IMMUNOTURBIDIMETRY

Tests Done : AAROGYAM CAMP PROFILE 2

Report Remarks : Labcode:1304111774/JRD32

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		Sample Type | Barcode	: SERUM | EV175169

TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	**IMMUNOTURBIDIMETRY**	**3.98**	**mg/L**

Bio. Ref. Interval. :-

< 1.00 - Low Risk
1.00 - 3.00 - Average Risk
>3.00 - 10.00 - High Risk
> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection , active arthritis or concurrent illness.

Clinical significance:
High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

Kit Validation Reference:
1.Clinical management of laboratory date in medical practice 2003-3004, 207(2003).
2.Tietz : Textbook of Clinical Chemistry and Molecular diagnostics :Second edition :Chapter 47:Page no.1507- 1508.

Please correlate with clinical conditions.
Method:- FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

Tests Done : AAROGYAM CAMP PROFILE 2

Report Remarks : Labcode:1304111774/JRD32

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		Sample Type | Barcode	: SERUM | EV175169

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
TOTAL CHOLESTEROL	PHOTOMETRY	98	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**26**	**mg/dL**	**40-60**
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	52	mg/dL	< 100
TRIGLYCERIDES	**PHOTOMETRY**	**174**	**mg/dL**	**< 150**
TC/ HDL CHOLESTEROL RATIO	CALCULATED	3.8	Ratio	3 - 5
TRIG / HDL RATIO	**CALCULATED**	**6.74**	**Ratio**	**< 3.12**
LDL / HDL RATIO	CALCULATED	2	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.49	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	71.92	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	34.74	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

Tests Done : AAROGYAM CAMP PROFILE 2

Report Remarks : Labcode:1304111774/JRD32

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MD(Path)

Dr.Amit Anand
DCP(Path)

Patient Name : NISHANT KUMAR (29Y/M)
Referred By : SELF
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Sample Type | Barcode : SERUM | EV175169

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ALKALINE PHOSPHATASE	PHOTOMETRY	74	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.52	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.1	mg/dL	0 - 0.20
BILIRUBIN (INDIRECT)	CALCULATED	0.42	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	15.8	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	**PHOTOMETRY**	**41.3**	**U/L**	**< 35**
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	30.1	U/L	< 45
SGOT / SGPT RATIO	CALCULATED	1.37	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	8.14	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.51	gm/dL	3.2-4.8
SERUM GLOBULIN	**CALCULATED**	**3.63**	**gm/dL**	**2.5-3.4**
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.24	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Diazonium salt DPD method
BILD - Diazonium salt DPD method
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg[1]method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

Tests Done : AAROGYAM CAMP PROFILE 2

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Sample Received on (SRT) : **13 Apr 2026 22:05**
Report Released on (RRT) : **14 Apr 2026 01:13**
Sample Type | Barcode : **SERUM | EV175169**

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	11	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.81	mg/dL	0.72-1.18
BUN / SR.CREATININE RATIO	CALCULATED	13.58	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	23.54	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	29.06	Ratio	< 52
CALCIUM	PHOTOMETRY	9.28	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	7.16	mg/dL	4.2 - 7.3

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

Tests Done : AAROGYAM CAMP PROFILE 2

Report Remarks : Labcode:1304111774/JRD32

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		Sample Type | Barcode	: SERUM | EV175169

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	C.L.I.A	86	ng/dL	60-200
TOTAL THYROXINE (T4)	C.L.I.A	7.7	µg/dL	4.5-12
TSH - ULTRASENSITIVE	C.L.I.A	2.298	µIU/mL	0.55-4.78

Comments :

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3 - Competitive Chemi Luminescent Immuno Assay
T4 - Competitive Chemi Luminescent Immuno Assay
USTSH - Third Generation Ultrasensitive Chemi Luminescent Immuno Assay

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Tests Done : AAROGYAM CAMP PROFILE 2

Report Remarks : Labcode:1304111774/JRD32

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TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	122	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease
<15 : Kidney Failure

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- 2021 CKD EPI Creatinine Equation

Tests Done : AAROGYAM CAMP PROFILE 2

Report Remarks : Labcode:1304111774/JRD32

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MC-5620

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Address	: PLOOT NO- 292, BARI CO-OPERATIVE COLONY, SIWANDIH, BOKARO STEEL CITY, JHARKHAND - 827010	**Report Released on (RRT)**	: **13 Apr 2026 23:30**
		Sample Type | Barcode	: **EDTA Whole Blood | EV175168**

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	H.P.L.C	4.9	%

Bio. Ref. Interval. :

As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	94	mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Tests Done : HBA PROFILE,HEMOGRAM

Report Remarks : Labcode:1304112799/JRD32

Dr Anupama Sinha
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Dr.Amit Anand
DCP(Path)

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Patient Name	: NISHANT KUMAR (29Y/M)	Sample Collected on (SCT)	: 13 Apr 2026 11:52
Referred By	: SELF	Sample Received on (SRT)	: 13 Apr 2026 22:19
Address	: PLOOT NO- 292, BARI CO-OPERATIVE COLONY, SIWANDIH, BOKARO STEEL CITY, JHARKHAND - 827010	Report Released on (RRT)	: 13 Apr 2026 23:30
		Sample Type | Barcode	: EDTA Whole Blood | EV175168

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	14.5	g/dL	13.0-17.0
Hematocrit (PCV)	**CPH Detection**	**52.7**	**%**	**40.0-50.0**
Total RBC	**HF & EI**	**5.59**	**X 10^6/µL**	**4.5-5.5**
Mean Corpuscular Volume (MCV)	Calculated	94.3	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	**Calculated**	**25.9**	**pq**	**27.0-32.0**
Mean Corp.Hemo. Conc (MCHC)	**Calculated**	**27.5**	**g/dL**	**31.5-34.5**
Red Cell Distribution Width - SD (RDW-SD)	**Calculated**	**56.4**	**fL**	**39-46**
Red Cell Distribution Width (RDW - CV)	**Calculated**	**16.2**	**%**	**11.6-14**
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	273.3	-	*Refer Note below
MENTZER INDEX	Calculated	16.9	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	6.72	X 10^3 / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	59.2	%	40-80
Lymphocytes Percentage	Flow Cytometry	34.8	%	20-40
Monocytes Percentage	**Flow Cytometry**	**1.8**	**%**	**2-10**
Eosinophils Percentage	Flow Cytometry	3.6	%	1-6
Basophils Percentage	Flow Cytometry	0.3	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	3.98	X 10^3 / µL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	2.34	X 10^3 / µL	1.0-3.0
Monocytes - Absolute Count	**Calculated**	**0.12**	**X 10^3 / µL**	**0.2 - 1.0**
Basophils - Absolute Count	Calculated	0.02	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.24	X 10^3 / µL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10^3 / µL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	157	X 10^3 / µL	150-410
Mean Platelet Volume (MPV)	**Calculated**	**13**	**fL**	**6.5-12**
Platelet Distribution Width (PDW)	**Calculated**	**19**	**fL**	**9.6-15.2**
Platelet to Large Cell Ratio (PLCR)	**Calculated**	**48.4**	**%**	**19.7-42.4**
Plateletcrit (PCT)	Calculated	0.2	%	0.19-0.39

Remarks : Alert!!! RBCs:Mild anisopoikilocytosis. Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

***Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

~~ End of report ~~

Tests Done : HBA PROFILE,HEMOGRAM

Report Remarks : Labcode:1304112799/JRD32

Dr Anupama Sinha
MD(Path)

Dr.Amit Anand
DCP(Path)

Scan QR to verify(valid for 30 days from release time)

CUSTOMER DETAILS

As declared in our data base

Name: NISHANT KUMAR **Age:** 29Y **Sex:** M

Barcodes/Sample_Type	:	EV175169(SERUM), EV175168(EDTA)
Labcode	:	1304111774, 1304112799
Ref By	:	SELF
Sample_Type/Tests	:	SERUM:AAROGYAM CAMP PROFILE 2 EDTA:HBA PROFILE, HEMOGRAM - 6 PART (DIFF)
Address	:	PLOOT NO- 292, BARI CO-OPERATIVE COLONY, SIWANDIH, BOKARO STEEL CITY, JHARKHAND - 827010
Sample Collected on (SCT)	:	13 Apr 2026 11:52
Report Released on (RRT)	:	14 Apr 2026 01:13
Amount Collected	:	-

CONDITIONS OF REPORTING

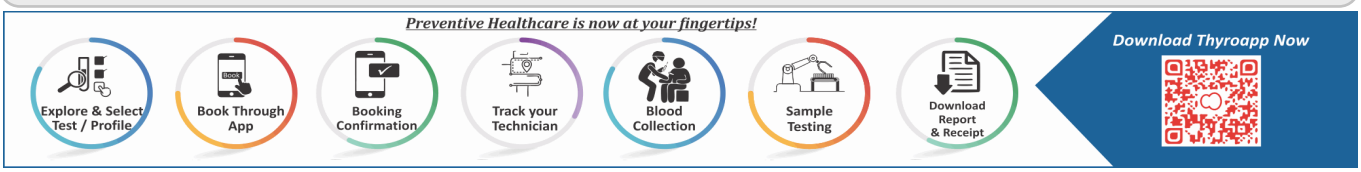
- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Neither Thyrocare, nor its employees/representatives assume: (a) any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report, (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.
- Thyrocare Discovery video link :- https://youtu.be/nbdYeRgYyQc

EXPLANATIONS

- Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- Testing or retesting should be done in accredited laboratories.
- For suggestions, complaints, clinical support or feedback, write to us at **customersupport@thyrocare.com** or call us on **022-3090 0000**



+ *T&C Apply, #As on 5th December 2024 (Applicable for all company owned labs except Bhagalpur & Vijayawada),*
* *As per survey on doctors' perception of laboratory diagnostics (IJARIIT, 2023), -Mumbai Reference Lab is CAP Accredited*